

DESIGNATED
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of integrated oncology and palliative care services

Quality matters in cancer care



BASAVATARAKAM INDO-AMERICAN CANCER HOSPITAL & RESEARCH INSTITUTE

Promoted by Smt. Nandamuri Basavataraka Ramarao Memorial Cancer Foundation
& Indo-American Cancer Organisation

Road No 10, Banjara Hills, Hyderabad 500034, India
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E-Mail : info@induscancer.com www.induscancer.com



DEPARTMENT OF NUCLEAR MEDICINE AND PET CT

Patient ID	: MR2507056	NM number	: PETCT - 0425351
Patient name	: Mr. TUMMALA CHENNA RAO	Bill number	:
Age / Gender	: 63 Y / Male	Visit ID	: DGMR250050953
Date & Time	: 16-Apr-2025 12:30 PM	Ref. Doctor	: Dr. ROHITH S

Clinical Diagnosis: Follow up case of carcinoma prostate; post radiotherapy (2021) and hormonal therapy; NET pancreas post 6 cycles of chemotherapy and radiotherapy (2025); now brain metastases. For evaluation.

PET-CT WHOLE BODY

Technique: Whole body CECT & PET scan was performed from vertex of the skull to mid thigh after injecting about 10 mCi of 18F FDG intravenously. Scan was acquired 60minutes post injection. Patients blood glucose levels were 192 mg/dl

Normal physiological 18F-FDG tracer uptake is seen in brain, pharyngeal tonsils, vocal cords, myocardium, liver, gut, kidneys and urinary bladder.

BRAIN:

- Increased FDG uptake in peripherally enhancing hypodense lesions in right frontal and left parietal lobes with mild to moderate perilesional edema in mild midline shift of measuring ~ 3mm, largest in left parietal lobe measuring ~5.2x3.1cm (SUV max 7.0).

HEAD & NECK:

- Naso, oro, hypo-pharynx and larynx are normal.
- No evidence of significant cervical lymphadenopathy.

CHEST :

- No significant FDG uptake in few small sub-centimeter sized mediastinal lymph nodes - reactive.
- Fibrotic changes in both lungs.
- Chest wall is normal.
- No pleural effusions.

ABDOMEN:

- No FDG uptake in bulky pancreatic head with no obvious soft tissue density lesion / FDG up suggested Ga68-DOTA PET CT.



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- Prostate measuring ~ 3.8x3.6x3.4cm with no obvious soft tissue density lesion / abnormal FDG uptake - suggested Ga68-PSMA PET CT
- Liver, gall bladder, kidneys, spleen are normal.
- No evidence of para aortic, paracaval, pelvic lymphadenopathy.
- No free fluid in the abdomen / pelvis.

BONES:

- No demonstrable lytic lesions.

IMPRESSION: Follow up case of carcinoma prostate; post radiotherapy (2021) and hormonal therapy; NET pancreas post 6 cycles of chemotherapy and radiotherapy (2025); now brain metastases, the present PET CT scan reveals:

- No FDG uptake in bulky pancreatic head with no obvious soft tissue density lesion / FDG uptake - suggested Ga68-DOTA PET CT.
- Prostate measuring ~ 3.8x3.6x3.4cm with no obvious soft tissue density lesion / abnormal FDG uptake - suggested Ga68-PSMA PET CT
- Hypermetabolic peripherally enhancing hypodense lesions in right frontal and left parietal lobes with mild to moderate perilesional edema and mild midline shift - likely to be metastases.
- No other evidence of metabolically active disease in present whole body scan.

Compared to previous PET CT dated 18/12/2024, there is:

- brain lesions are new.
- no significant interval change in other findings.

Note: Whole Body FDG PET-CT is not very sensitive for brain metastasis.

Dr. ZAKIR ALI A

Sr. Consultant Nuclear Medicine

Date: 16-Apr-2025 12:30 PM

Dr. PRATHYUSHA B

Consultant Nuclear Medicine and PET

16-Apr-2025 03:37 PM

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BASAVATARAKAM



DISCHARGE SUMMARY

Patient name : Mr. TUMMALA CHENNA RAO
Visit ID : RTMR250002088
Age / Gender : 63 Yrs, 0 M, 11 D / Male
Admission date : 18-04-2025 1:47PM

Patient ID : MR2507056
Patient category : Cash
Bed / Nursing station : N/A
Discharge date : 26-04-2025

Consultant(s)

Dr ROHITH S DNB(RT)

Final Diagnosis: Carcinoma Prostate and Ca Pancreas with Brain Mets

Case History: known case of Carcinoma Prostate received radical radiotherapy from 26/2/2021 to 2/4/2021, last ADT on 10/4/2024 then diagnosed with Ca Pancreas received NACT 6 cycles last on 14/11/2025 followed by radical radiotherapy 56Gy in Feb 2025. Now presented with c/o headache since 2 months. MRI showed 2 cystic brain lesions hence planned for SRS to brain lesions.

Investigations: Whole Body PET CT on 16/4/2025 - Follow up case of carcinoma prostate: post radiotherapy (2021) and hormonal therapy; NET pancreas post 6 cycles of chemotherapy and radiotherapy (2025); now brain metastases, the present PET CT scan reveals: No FDG uptake in bulky pancreatic head with no obvious soft tissue density lesion / FDG uptake - suggested Ga68-DOTA PET CT. Prostate measuring ~ 3.8x3.6x3.4cm with no obvious soft tissue density lesion / abnormal FDG uptake - suggested Ga68-PSMA PET CT Hypermetabolic peripherally enhancing hypodense lesions in right frontal and left parietal lobes with mild to moderate perilesional edema and mild midline shift - likely to be metastases. No other evidence of metabolically active disease in present whole body scan.

Compared to previous PET CT dated 18/12/2024, there is:

- i) brain lesions are new.
- ii) no significant interval change in other findings.

Case Discussion: After explaining the prognosis, benefits and side effects of radiation to patient and patient attenders. He is planned for radiation to 2 brain lesions site by SRS technique on TOMOTHERAPY.

CT DONE ON: 18/4/2025

RT START DATE: 22/4/2025

RT END DATE: 26/4/2025

TOTAL DOSE: 25Gy

DOSE PER FRACTION: 5Gy

NO OF FRACTIONS: 5

Tolerated treatment well

Condition of Discharge: Stable

Discharge Advice: Review after 2 months

Report Immediately to the hospital in case of Fever/ Diarrhea/ Excessive Vomiting/ Breathlessness. In case of

Emergency contact Ph: 040-2355 1235. Emergency Room (Ext - 2499)

Discharge summary received by

Signature:

Name:

Relation:

Dr. ROHITH S
Registrar (Consultant)
Reg. No. 70874
Basavatarakam Indo-American
Cancer Hospital & Research Institute
Road No: 10, Banjara Hills,
Hyderabad - 500 034. T.G.